

Eleanor Vance

Age : 54 Years

Sex : Female



PID : 882

Apt ID : 2026001

Ref. By : Dr. Arthur Davies

Study Date & Time:

09:15 AM 15 Sep, 2026

Report Date & Time:

09:45 AM 15 Sep, 2026

BRAIN MRI WITH CONTRAST**Brain MRI - Axial T1ce & FLAIR View**

- Heterogeneously enhancing large intra-axial mass identified in the right frontoparietal parenchyma measuring approximately 4.8 x 4.2 cm.
- Central non-enhancing liquefactive necrosis with irregular, thick peripheral hyper-enhancing ring margins.
- Marked perilesional vasogenic edema extending along adjacent subcortical white matter tracts and corona radiata.
- Significant positive mass effect with compression and near-total effacement of the ipsilateral right lateral ventricle.
- Subfalcine herniation with 4.5 mm midline shift of cerebral structures toward the contralateral left hemisphere.
- Flow voids in the major circle of Willis intracranial arteries appear preserved.

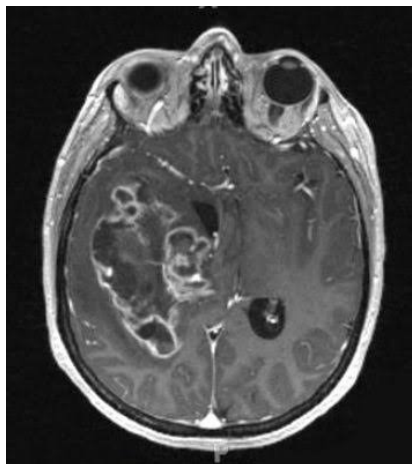
DISEASE IDENTIFIED:	Primary Malignant Brain Neoplasm (Brain Tumor)
TUMOR / SUB-TYPE:	Glioblastoma Multiforme (GBM) - CNS WHO Grade IV (IDH-wildtype Astrocytoma)
STAGE / PHASE:	Advanced Necrotic Phase (WHO Grade IV) with Mass Effect & 4.5mm Midline Shift

IMPRESSION

Right frontoparietal necrotic ring-enhancing mass with extensive surrounding vasogenic edema and 4.5 mm midline shift, pathognomonic for High-Grade Glioma / Glioblastoma Multiforme (WHO Grade IV).

ADVICE

Urgent neurosurgical consultation for stereotactic navigation-guided craniotomy and debulking; adjuvant Stupp protocol (concurrent Temozolomide + Radiotherapy).



Thanks for Reference

****End of Report****

Radiologic Technologists

(MSC, PGDM)

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Dr. Payal Shah

(MD, Radiologist)

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